Louisiana Treatment Centers

Capital Requirements & Staged Entry

Prepared: 6 October 2026

Revision: 3 — post-adversarial-review

Status: Capital requirements and staged entry analysis

Scope: Business and regulatory analysis for private planning. Not legal, clinical, tax, or investment advice. Capital figures are build-up estimates from actual Louisiana LDH licensing fees and Medicaid rates (IB 25-31, eff. 1/1/2026) combined with directional national ranges for build-out, insurance, and staffing — they are planning estimates, not quotes. Section 8 lists every figure requiring independent confirmation before capital is committed.

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Louisiana Treatment Center Venture — Capital Requirements & Staged Entry

Prepared: 2026-10-06 · **Revision 3.** An adversarial review overturned several revision-2 conclusions, including two I had stated as findings. Corrections are marked inline, because the wrong versions were circulated.

> ***Source discipline.*** *Figures are tagged by provenance. [vendor] means the number comes from a firm that sells licensing, accreditation, or brokerage services and monetizes complexity — treat those as unverified anchors, not facts. Regulatory and rate figures are from primary sources.*

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0. Direct answer: what capital is required

Entry point	Build + licensing	Required reserve (6–12 mo opex)	All-in	Time to first dollar
Recovery residence, per 10-bed house	$25–50K	$20–35K	**$50–85K**	30–60 days
3 recovery residences	$75–150K	$60–105K	**$150–255K**	60–120 days
ASAM 2.1 IOP (licensed outpatient)	$150–300K	$250–400K	**$400–700K**	12–18 months
ASAM 3.1 residential	$200–400K	$300–500K	**$500–900K**	18–24 months
ASAM 3.5 residential	$350–700K	$550K–1.3M	**$900K–2.0M**	18–30 months
ASAM 3.7 / 3.7-WM withdrawal management	$600K–1.2M	$900K–1.8M	**$1.5–3.0M+**	24–36 months
Acquire existing licensed outpatient	3–5× EBITDA [vendor]	+$150–300K transition	**purchase + reserve**	6–12 months

The dominant line is not build-out — it is the pre-revenue window. For any licensed level you are fully staffed, fully compliant, and collecting nothing for 6–12 months through licensure and payer credentialing. In the IOP model that reserve is **$250–400K of the $400–700K.** Ring-fence it.

Minimum to begin credibly: $150–255K. To reach a licensed clinical business: $400–700K for a de novo IOP — the cheapest licensed entry and the one Louisiana's rates reward best.

0.1 Three corrections to what I told you in revision 2

1. **"Louisiana requires no Certificate of Need" — wrong as stated.** Louisiana has no traditional CON, but its **Facility Need Review (FNR)** program gates BHSP licensure for **PSR, CPST, and OTP** services: "no BHSP or applicants seeking to provide psychosocial rehabilitation (PSR) and/or community psychiatric support and treatment (CPST) services shall be eligible to apply for licensure … unless the FNR committee has granted FNR approval" [LAC §12 525(A)]. **SUD residential (3.1/3.3/3.5/3.7) and IOP are not FNR-gated** — so your core plan is clear. But if you add CPST/PSR (the high-volume Medicaid outpatient lines) or opioid treatment/MAT, FNR approval is a precondition to *applying*.

2. **"Accreditation is a licensing condition for residential" — wrong.** LAC Ch. 56 contains **no accreditation prerequisite for licensure of any service type.** Accreditation appears only as optional *Deemed Status* (accepted "in lieu of on-site licensing surveys") and as a revocation ground "if accreditation is a federal or state requirement for participation in the program" [LAC Ch.56]. The actual requirement is **Medicaid enrollment**: facilities "must be accredited prior to enrollment" by CARF, COA, or TJC [LA Medicaid BHS Provider Manual]. **A commercial/private-pay-only residential program may be licensable without accreditation.**
3. **My "accreditation catch-22" was overstated — there is a defined pathway.** CARF requires six months of *conformance/service records by the survey date*, but new programs not yet serving clients can obtain **Preliminary Accreditation** by presenting proposed treatment and operational procedures, with a full survey roughly six months after services begin. The real sequence is: license → open → Preliminary Accreditation → operate 6 months → full survey → Medicaid enrollment. Not circular. Confirm specifics with CARF and LDH.

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1. The Louisiana numbers that actually drive the model

1.1 State licensing fees are trivial; the cost is everything around them

[LDH HSS fee schedule] BHSP initial license **$600**; renewal $600; $5/unit; offsite/branch $300; **CHOW $600**; change of address $600. A statutory escalator (HB 414, 2017 RS) trends fees toward a $1,000 cap.

National guides quoting "$25,000–$100,000 for licensing and legal" [vendor] are describing **attorneys, policy development, licensing consultants, and accreditation prep — not state fees.** Budget that work honestly; do not mistake the $600 for the cost of getting licensed.

1.2 Actual Medicaid rates, effective 1/1/2026

[IB 25-31 rev. 11/5/2025]. Adult rates. Credential tiers: **HO** = Master's, **HN** = Bachelor's, **HM** = less than Bachelor's.

Code	Level	Service/day	R&B/day	All-in
H0015	**IOP 2.1, group**	**$180 / $150 / $120** (HO/HN/HM)	—	per day
H2034	3.1 halfway house (18+)	$120.26	$14.70	$134.96
H0019	3.3 long-term residential	$104.38	$21.50	$125.88
H2036	**3.5 clinically managed high-intensity (18+)**	**$303.67**	$31.62	**$335.29**
H2036	3.7 medically monitored (21+)	$404.13	$56.26	$460.39
H0011	3.7-WM acute withdrawal management (21+)	$425.61	$43.50	$469.11

Code	Level	Service/day	R&B/day	All-in
H0001	Assessment (18+)	$81.59 / $79.81 / $78.03	—	per visit
H0004	Individual counseling (18+)	$18.57 (HO)	—	per 15 min

Correction to revision 2: I used $364.24 for ASAM 3.5 — that is the *prenatal/postpartum* variant. The **general adult 3.5 rate is $303.67.**

⚠ **Room and board is MCO-discretionary.** [IB] states R&B rates "have been removed from the SBHS fee schedule, as these are not State Plan services and are payable at the discretion of the managed care organization." Plan on **service-only** rates; treat R&B as upside to be confirmed MCO-by-MCO.

Three consequences:

(a) Staff credential mix is a revenue decision. IOP group pays **$180/day at Master's level versus $120 at less-than-Bachelor's** — a 50% revenue swing on identical service. Cheaper staff can cut revenue faster than payroll.

(b) Louisiana residential is more viable than revision 2 implied. At 24 beds, 80% occupancy, service-only: **$303.67 × 24 × 0.80 × 365 ≈ $2.13M/yr** (≈$2.35M if R&B is paid). Revision 2's "$150–300 Medicaid" range bracketed neither end — 3.1/3.3 fall below it, and 3.5/3.7 (the levels an actual residential program bills) fall well above. The commercial-vs-Medicaid spread is roughly **2.4–4.5×**, not the 4–5× I stated. **This weakens the case for leaving Louisiana.**

(c) IOP remains the best capital efficiency, but on throughput, not per-diem comparison — see §2.2.

1.3 ASAM 4th Edition: rulemaking is complete

[LDH HSS memo 2/4/2026] Notice of Intent published 9/20/2025; **final rule published in the Louisiana Register 1/20/2026**; "all BHSPs shall be required to be in compliance with the updated regulations no later than **July 1, 2026**." This answers revision 2's open question — the rules are final, not pending.

Terminology follows: under 4th Edition, former "PHP / Level 2.5" is **High-Intensity Outpatient**, Level 1 becomes **1.5**, and the 3.x/WM levels are restructured. Build to 4th Edition labels.

1.4 Incumbent operators are publicly warning

Leaders from Odyssey House Louisiana and O'Brien House testified that rate levels and eligibility gaps together threaten provider capacity [trade]. The 2025 legislature did fund an increase (implemented as the higher of new actuarial rates or the existing temporary 25% bump) [IB]. Weigh accordingly: do not enter Medicaid-only at thin margins with no commercial or private-pay mix.

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2. Capital models

2.1 Recovery residence (unlicensed), 10 beds

Line	Amount
Lease deposit + first/last (5–6 BR, LA)	$6,000–10,000
Furnishings	$15,000–25,000
Entity formation + legal	$3,000–8,000
Insurance (GL, property, yr 1)	$3,000–6,000
NARR affiliate certification	$500–2,000
Working capital, 6 mo partial occupancy	$20,000–35,000
Total per house	**$50,000–85,000**

Stabilized: 10 beds × $175–250/wk = $7,600–10,800/mo gross; net **$400–3,500/mo per house.** Three to five stabilized houses is **$1,200–17,500/month** — a band so wide it conveys little, and whose low end is a poor return on 18 months. **No LDH license is required** provided no clinical service is offered; the moment clinical services are added you are a BHSP and must be licensed. Draw that line in writing with counsel.

2.2 ASAM 2.1 IOP — the recommended licensed entry

Line	Amount
LDH BHSP initial license [LDH]	$600
Healthcare regulatory counsel (entity, CPOM, EKRA, Part 2, payer contracts)	$25,000–50,000
Policy/procedure development + licensing consultant [vendor]	$15,000–40,000
Lease deposit + build-out, 2,500–4,000 sq ft [vendor]	$40,000–120,000
Furnishings, IT, EMR	$20,000–40,000
Professional liability (see §4.3 — availability, not price, is the gate)	$15,000–40,000
General liability + abuse/molestation + D&O + cyber/Part 2 breach	$10,000–25,000
CARF accreditation incl. prep [vendor]	$25,000–75,000
Credentialing consultant	$10,000–25,000
Marketing, website, LegitScript	$5,000–15,000
Pre-revenue staffing, 9–12 mo (Clinical Director $85–110K; 2 counselors $46–62K ea.; admin $35–45K; Medical Director stipend $2–5K/mo)	**$250,000–400,000**
Contingency 15%	$55,000–95,000
Total	**$400,000–700,000**

Revenue — modeled on throughput, not a 52-week multiplier. Revision 2 computed "25 clients × 3 sessions × 52 weeks," which assumes every client stays a year. IOP episodes typically run 6–12 weeks, so revenue is governed by **admissions throughput × authorized length of stay**:

Admissions/mo	Avg LOS	Steady census	Annual (3 sessions/wk @ $180)
8	8 weeks	~15	~$420K
12	8 weeks	~22	~$620K
12	10 weeks	~28	~$780K
16	10 weeks	~37	~$1.04M

Payer-authorized length of stay is the variable that controls this, and it is not under your control. At ~22 steady census and 15–25% operating margin, indicative EBITDA is **$95–155K** on $400–700K deployed.

2.3 Residential 3.1 / 3.5

Line	3.1	3.5
Property: deposit + licensing-standard build-out [vendor]	$100–250K	$200–500K
Legal, policy, licensing consultant	$40–90K	$50–120K
Accreditation (**for Medicaid enrollment**, not licensure — see §0.1)	$35–85K	$40–100K
Furnishings, IT, EMR	$30–60K	$40–90K
Insurance (all lines; residential is materially harder to place)	$25–55K	$40–90K
Pre-revenue staffing incl. 24/7 coverage, 9–12 mo	$300–500K	$550K–1.3M
Contingency	$60–120K	$120–250K
Total	**$500–900K**	**$900K–2.0M**

At 24 beds / 80% / service-only $303.67 → **~$2.13M/yr** revenue at 3.5.

Note on "inpatient." Your stated goal says inpatient. In Louisiana, ASAM residential levels sit under **BHSP licensure**; true psychiatric-hospital inpatient is a **separate hospital licensure category** and a materially harder path. This plan addresses the former.

2.4 Acquisition — corrected rationale

You do not buy the license. [LAC Ch.56] on CHOW: the new owner must submit "**all documents required for a new license**" plus the fee, and "once all application requirements are completed and approved … **a new license may be issued to the new owner**." The license is valid up to one year and for one geographic service area only.

And the distressed-asset angle is largely closed: "**A BHS provider that is under license revocation, provisional licensure or denial of license renewal may not undergo a CHOW**" [LAC Ch.56]; outstanding fees or fines suspend a CHOW until paid. The struggling sellers a bargain-hunter would target are frequently the ones legally barred from selling.

What an acquisition does buy: physical plant already built and surveyed to standard; clinical leadership and staff in post; existing census and referral relationships; **accreditation**, which attaches to the program; policies that have passed survey; documented operating history; and a seller available for transition. Those are real. **Budget $150–300K transition reserve** for CHOW re-licensure, re-credentialing, and census disruption. Payer credentialing generally attaches to entity/TIN — assume Medicaid re-enrollment and check each commercial contract for assignment/novation clauses rather than assuming they travel.

2.5 Financing — and the downside

- **SBA 7(a)** finances startups and acquisitions to $5M. **Correction to a common belief: 13 CFR § 120.150**, as amended at 88 FR 21085 (Apr. 10, 2023) [CFR], lists only credit history, earnings/cashflow, and equity/collateral — the former "experience and depth of management" criterion **was removed.** Industry experience is now *lender credit policy*, not an SBA eligibility rule. In licensed healthcare, lenders commonly still require relevant management experience plus evidence licensure will be reissued. **The binding constraint on a Louisiana acquisition is the state CHOW, not SBA eligibility.**
- **The downside, stated with the same specificity as the upside:** 7(a) loans are personally guaranteed by owners of 20%+, commonly secured against home equity. A $1–3M acquisition means a **$1–3M personal guarantee.** Combined with the undercapitalization risk, the realistic bad outcome is not "lose the invested capital" — it is **lose the capital, owe the guarantee, and carry it personally after the entity fails.**
- SBA's **active-ownership rule** bars passive investors: an "own it, hire an administrator, stay employed in tech" structure is not 7(a)-financeable.

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3. Sequencing — revised after review

Revision 2 recommended *sober living → IOP → residential*. The review substantially damaged the first leg, and I now think it was wrong.

The case against sober-living-first, which I accept:

4. **It does not build the track record that matters.** What a BHSP applicant and a payer need is *clinical*: ASAM level-of-care fidelity, LMHP supervision structure, treatment planning and documentation, utilization review, survey response, incident reporting, 42 CFR Part 2 handling. A recovery residence produces none of these — it is a leased-property business with a house manager.
5. **It manufactures your largest criminal exposure.** An owned sober-living → owned-treatment referral path is the paradigm EKRA "body brokering" fact pattern. Sequences that put IOP first simply do not create the structure. This is a *cost of the inversion*, not a neutral feature.
6. **Its own economics are weak** — $1,200–17,500/month across 3–5 houses for 18 months of work.

What survives from revision 2: the *ordering* of difficulty. Entering at residential or detox first — no clinical credential, no operating history, no payer contracts, no survey record — remains the worst available entry. That conclusion does not depend on any contested number.

The revised option set, including the one I omitted

Option	Capital at risk	What it buys	Honest verdict
A. Status quo — stay employed, hold assets as options	$0	Preserves baseline; everything stays open	**Omitted from revision 2. That was a framing failure**, and my sibling memo named the same failure a week's work earlier. It belongs on the list.
B. Paid industry employment — program administrator / director of operations / payer-contracting lead at an existing LA program, 18–36 months	**Negative** (you are paid)	All three deficits §4 names: clinical credibility, payer-contracting experience, survey track record. Plus the local referral web and Clinical Director relationships.	**Strongest risk-adjusted path to ownership.** Delays it; materially de-risks it. Also an insurance-underwriting input (§4.3).
C. De novo IOP	$400–700K	A licensed clinical business	Correct first *owned* step. Best after B.
D. Acquire licensed outpatient	Purchase + $150–300K	Plant, staff, census, accreditation, history — **not** the license	Viable; rationale corrected; distressed targets largely barred from CHOW
E. Recovery residences	$50–85K/house	Cash flow, referral web, Regroup in production	Fine as a **side activity under separate ownership**, papered for EKRA. Not a stage.
F. Residential first	$900K–2.0M	—	**Do not.**

Recommended sequence: B → C → residential, with E optional alongside under genuinely separate ownership and EKRA structuring in place before the second entity exists.

The benchmark this must clear. Your prior corporate-engineer compensation is recorded at **~$145K** in the sibling analysis. Against that: Stage E is $18–210K/yr across 3–5 houses; a stabilized IOP is ~$95–155K EBITDA. **The owned path spends 12–36 months and six figures of capital to arrive at roughly the income you already have, with capital at risk and criminal-regulatory exposure added.** The ceiling above that is real — a stabilized 3.5 residential program at ~$2.1M revenue and 20–30% margin is $400–640K EBITDA — but it is Stage 3, not Stage 1.

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4. Non-negotiable architecture

4.1 You are not a clinician

A Louisiana BHSP license is **institutional** (held by the entity), so non-clinician ownership is legal and common — unlike a dental or medical practice, where the license is personal. You must install a **Medical Director** (physician, typically part-time stipend) and a **Clinical Director** (independently-licensed LMHP). For SUD,

Louisiana requires an identified **Addictionologist** (§5693) and sufficient independently-licensed LMHPs [LAC]. **The Clinical Director is your single most important hire.**

4.2 EKRA

[18 U.S.C. § 220] — "shall be fined not more than **$200,000**, imprisoned not more than **10 years**, or both, **for each occurrence**." Covers recovery homes, clinical treatment facilities, and laboratories, and is not limited to federal programs.

United States v. Schena (9th Cir. 2025): **the court affirmed the EKRA conviction.** Its narrow holding is that percentage-based compensation is not *per se* unlawful absent undue influence over the flow of referrals. Read it as a conviction affirmed with a narrow carve-out — not as reassuring. It binds only the Ninth Circuit.

Build in from day one: no admissions, marketing, or outreach compensation varying with headcount, census, or revenue — **salary only**, with contemporaneous documentation. **42 CFR Part 2** governs SUD records more strictly than HIPAA and reaches your software.

4.3 Insurance — availability is the gate, not price

Break out and quote early: **professional liability**, general liability, **abuse/molestation** (frequently sublimited or excluded in behavioral health), D&O, and **cyber / Part 2 breach**. Carriers underwrite on operator experience, level of care, and claims history; residential and withdrawal management are materially harder and dearer than outpatient. **A declination is a stop signal**, and operating history is an underwriting input — another argument for Option B.

4.4 Wind-down and continuity of care

Absent from revision 2 entirely, in a sector with accelerating closures. [LAC Ch.56]: a license "shall be immediately null and void if a BHS provider ceases to operate," and "during the period of inactivation, the department prohibits CHOW" — **an operator in trouble cannot simply sell out of it.** Clinical staff carry individual licensure duties that survive the LLC's cash.

Gate each stage on: transfer agreements with at least two competitor programs; a ring-fenced closure reserve covering 30–60 days of care continuity; a named clinical lead accountable for orderly transfer.

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5. Risk register

Risk	Severity	Note
Underfunding the pre-revenue window	**Critical**	$250–400K of the IOP model alone
EKRA exposure from an owned sober-living→treatment continuum	**Critical**	$200K/10yr per occurrence; avoid by sequencing, not just papering
CHOW: license does not transfer; distressed sellers barred from CHOW	**Critical**	Was "High" in rev 2 — it erases the rationale for 3 of 5 capital tiers
Personal guarantee survives entity failure	**Critical**	$1–3M, commonly secured on home equity

Risk	Severity	Note
Residential R&B is MCO-discretionary	**High**	Plan service-only; confirm per MCO
Payer-authorized length of stay governs IOP revenue	**High**	Not under your control
Clinical Director hire fails	**High**	The #1 hire
Professional liability declination / abuse-molestation exclusion	**High**	Availability, not price
Patient/resident abandonment on failure	**High**	License void on ceasing operation; CHOW barred during inactivation
Staff credential mix erodes revenue	**High**	$180 vs $120/day, identical service
FNR gating if CPST/PSR/OTP enter scope	**High**	Precondition to *applying* for licensure
Medicaid-only payer mix at thin margins	**High**	LA incumbents warning publicly
Zoning denial / neighborhood opposition	Medium	Written confirmation before non-refundable money
Workforce turnover	Medium	**~33.2% measured** annual counselor turnover; field estimates 19–50% [lit: Eby & Rothrauff-Laschober 2010] — not the "30–50%" I cited
Opportunity cost of forgone salary	Medium	~$145K/yr baseline
Recovery-residence closure base rate	Medium	My sibling memo treated this as disqualifying for the software business; it applies equally to becoming one

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6. Next steps — free, high-information, no capital

7. **Louisiana healthcare-regulatory attorney**, scoped consult: entity structure, EKRA-safe compensation, and whether to run a continuum at all given §3's analysis.
8. **Confirm with CARF** the Preliminary Accreditation pathway and timeline for a new program, and with **LDH** whether licensure for your intended service types requires accreditation at all (per §0.1 it should not).
9. **Pull the authoritative Specialized Behavioral Health Services Fee Schedule.** [IB] excerpts are labeled informational only. (LDH blocks automated download — request it directly.)
10. **Ask two or three LA MCOs** how they handle residential room and board, and typical authorized length of stay for 2.1 IOP. The second answer drives the §2.2 revenue model.
11. **Explore Option B seriously** — talk to two or three LA operators about an operations or business-development role. This is the cheapest acquisition of the assets you lack, and it pays.
12. **Get indicative insurance quotes** for your intended level of care. A declination now saves six figures later.
13. **Begin the Clinical Director search informally** — to learn market rate and whether you can attract someone good. If you cannot, that is decisive early.

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7. Verify before relying

Not independently verified — treat as unverified anchors: startup cost ranges by level of care; the $800–1,500 commercial per diem; 3–5× / 5–8× multiples and the 0.5–1.0× accreditation premium; $25–75K accreditation prep; credentialing 90–180 days; the 114,000-counselor shortage projection; the $50–200K zoning-loss figure. All are [vendor]-class.

Also open: the authoritative fee schedule (§6.3); per-MCO R&B treatment; whether LA MCO contracts are assignable on CHOW; the *Schena* certiorari petition status; **Texas entirely** — if Texas is live, none of the licensing, CPOM, FNR, or rate analysis here transfers, and no Texas case has been made.

Corrected since revision 2: the CON claim (§0.1); accreditation as licensure condition (§0.1); the accreditation "catch-22" (§0.1); the ASAM 3.5 rate (§1.2); the Medicaid spread (§1.2); turnover (§5); SBA experience requirement (§2.5); the sober-living-first recommendation (§3).

7a. Updates from the capital-raising research

Three items were refined by the companion CAPITAL-RAISING-PLAN-2026-10-06:

14. **SAMHSA was not dissolved.** Congress rejected the FY26 proposal (Appropriations vote 26-3 against, with funding added) and approved a $7.1B topline; the Administration for a Healthy America was never authorized or funded. HHS has nonetheless proceeded administratively via staffing cuts and transferred duties, and the FY27 request renews the proposal.
15. **Louisiana's corporate-practice posture is softer than Section 4.1 implies.** The LSBME's December 2024 Statement of Position holds that physician employment by a non-professional corporation is **not per se unlawful**; the operative test is whether the arrangement impairs independent medical judgment. This materially simplifies structuring for a non-clinician owner relative to strict-CPOM states such as California and New York.
16. **Financing independently reinforces the Option B sequencing recommendation.** SBA removed "experience and depth of management" as a regulatory criterion in 2023 (13 CFR 120.150, 88 FR 21085), but lenders retained it as credit policy and in licensed healthcare commonly require relevant management experience plus evidence licensure will be reissued. A paid industry role therefore improves financeability on its own terms, independent of the operating-skill rationale.

Also newly identified there: an SBA-financed Louisiana acquisition has a **closing collision** - lenders fund at close and cite licensing transfers as a top delay source, while a Louisiana BHSP license does not transfer and must be reissued by LDH to the new owner. Condition closing on CHOW approval or escrow against it.

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8. Bottom line

Capital: $150–255K to begin with residences; **$400–700K for a licensed IOP**; $900K–2.0M for full residential. Roughly 60% of the IOP figure is pre-revenue payroll.

Three things that should change your plan:

17. **Do not start at residential** — that conclusion survived every correction.
18. **Do not start at sober living either.** It does not build clinical track record, it manufactures your largest criminal exposure, and its economics are thin. **Paid employment inside an existing Louisiana program is the cheapest way to acquire the three things you actually lack**, and it pays you rather than costing you.
19. **Louisiana is better than I said.** At the real ASAM 3.5 rate of $303.67/day service-only, a 24-bed program at 80% occupancy is ~$2.13M/yr. The commercial-vs-Medicaid spread is ~2.4–4.5×, not 4–5×. The case for leaving the state is weaker than revision 2 claimed — and no case for Texas has been made at all.

The honest frame: against a ~$145K baseline, the owned path spends 12–36 months and six figures to reach roughly your current income, with capital at risk, a personal guarantee, and criminal- regulatory exposure added. The ceiling above that is genuinely large — $400–640K EBITDA at a stabilized 3.5 program — but it sits two stages past the first dollar. Both facts are true at once, and the sequencing in §3 is the cheapest way to find out whether the ceiling is reachable before the capital is irrecoverable.